

LILLY (ELI) & CO LLY

July 18, 2026 - 1:06pm EST

by

[gary9](#)

			2026	2027
Price:	1,179.00	EPS	35	45
Shares Out. (in M):	941	P/E	33	26
Market Cap (in \$M):	1,100K	P/FCF	0	0
Net Debt (in \$M):	40,000	EBIT	0	0
TEV (in \$M):	1,140K	TEV/EBIT	0	0

Description

Eli Lilly (LLY) trades at 33x this year's earnings and 26x next year's. Other "big pharma" averages about 12x EPS.

So, obviously, you have to believe in certain things about LLY to justify such a premium and own it. You have to:

- love the mission,
- expect a sustainable growth rate well above average,
- see lots of potential for upside surprises,
- and believe in a positive "flywheel" that can perpetually refresh the portfolio as a durable wide-moat healthcare network.

I believe all these things present with LLY, plus likely to continue, and I am pleased to own it here. While the stock is certainly up a ton since GLP-1s arrived, we are only at the very dawn of pill form incretins and there is a massive market yet to be claimed. (Caveat: This is a high-level thesis presented by a generalist, non-expert in healthcare. Disclaimers like at the end of a pharma ad should roll here. Feel free to consult more granular experts and go easy on the messenger. Thank you.)

First, the mission. GLP-1s look like a godsend panacea for myriad amazing healthcare outcomes. What started with diabetes treatment (quite a large market to begin with - \$35 billion US, \$100B+ globally) has been reformulated in low dose for mass-market weight loss. And we are only in the early innings of development and adoption. Approximately 12% of US adults are taking GLP-1s today and only about 1% globally. Arguably, the market for weight management spans at least 30% of the global population. And applications beyond weight loss, such as addiction control and overall "wellcare" uses, we are just starting to hear more about.

Lilly has been solidifying its lead and preparing to dominate the future of incretins as we move from injectables to pill form. With the existing injectables, its "dual-acting" tirzepatide (commercialized as Mounjaro for Type-2 Diabetes and Zepbound for obesity) is clearly more efficacious than Novo's single-acting semaglutide Wegovy/Ozempic. I believe it's just an "early-inning" phenomenon that Ozempic has been the "brand name" rather than Mounjaro or Zepbound. Early in the game, endocrinologists have been swayed by Novo's marketing. Longer term I see them overwhelmingly recommending LLY's injectables over NOVO's, because they simply work better.

Also, we are in the very early innings of pill form incretins. Pill form undoubtedly will expand the market and accelerate penetration while also extending patent protections. In addition, pill form is potentially higher margin being much lower cost to manufacture and largely overcomes the supply constraints that constrained the growth of injectables. Again, I think it is pure noise that analysts are saying LLY's pill Foundayo had an "underwhelming launch" compared to Wegovy's pill. LLY's higher efficacy products are set to dominate the market. And of course we are getting closer to the approval and launch of Retatrutide - the first triple-agonist (targeting the gut, the brain and glucagon receptors) which has game-changer written all over it. Lilly should be in a dominant position for a very long time. There will always be cheaper solutions and off-price copycats, but there is much value to accrue to the market leader as the world conspires to take global adoption from 1% toward 30%.

It's important to love the mission in investing if you hope to stay convicted through volatile market swings and see your thesis through to fruition. This is such a socially-positive mission to finance great healthcare outcomes - way beyond just weight control. That of course doesn't guarantee a great stock outcome - competition, reimbursements, patent cliffs, regulatory change all have to be monitored and understood - yet, it's hard not to root for the LLY upside case to manifest.

Rapid growth and potential for upside surprises. Lilly is on track for \$85B revenues and \$35 eps in 2026. No one doubts both revenue and earnings will double ahead though we can quibble about whether it takes 4 years or plus or minus. Either way, I'm betting LLY will grow faster than the market for the modelable future. Thus, assuming a decent long-term growth rate is sustainable (never a sure bet in pharma), LLY should command and maintain an above-market multiple.

So how can we expect LLY to buck the trend of other pharma peers and maintain a sustainable long-term growth rate? First, looking at the core incretin franchise, I see much potential for upside surprises. The modeling is the usual conservative and linear guess work of sell-side analysts. Linear thinking can often miss dramatic S-curve adoption and resultant exponential growth. It's intriguing here to take the "over" on analyst expectations. I find the assumptions on ex-US growth and profits particularly cautious and backward-looking - based on a history of a ROW that can't pay and allows lawless and profitless drug piracy as the norm.

Beyond incretins, LLY has a strong diversified legacy portfolio and numerous potential blockbusters in development. Most promising is LLY's breakthrough Alzheimer's therapeutic, donanemab, showing the ability of management to bring drugs from conception stage to major market share competitors in critical markets.

And perhaps you have heard that AI has the potential to supercharge drug development (another exciting investible theme). Who do we think is best positioned to capitalize on that theme? Of course, LLY with the scale and stock currency to acquire the most promising franchises. CEO Dave Ricks has done a phenomenal job allocating capital for LLY, distancing his company from peers wrestling with patent cliff issues. In the first half 2026 alone, LLY has committed over \$20B to outright acquisitions, private company financings, licensing agreements and strategic partnerships in promising fields as diverse as oncology, genetics, pain management and infectious disease control. This activity that no competitor of LLY can match goes a long way toward ensuring a robust and growing portfolio in the future.

And then there is LillyPod. LLY and Nvidia have partnered to build the most powerful supercomputer ever owned and operated by a pharmaceutical company. LillyPod is part of a broader five-year, \$1 billion partnership between the two companies. They are establishing a co-innovation lab in San Francisco that physically co-locates Lilly's biologists and chemists with Nvidia's top AI engineers.

It is this beneficial portfolio-refreshing "flywheel" - powered by a lead position in incretins and the tailwind of unprecedented drug development - that can justify a sustainable differentiation in valuation between LLY and the rest of pharma competitors.

Basic numbers for LLY look like this:

	LLY	Street Estimates	2026	2027	2028
Price	1179	Revenue	85,000	99,000	111,000
Shares	941	EBITDA	41,500	52,600	60,700
Market Cap	1,109,439	Net Income	31,000	40,000	46,000
Net Debt	40,000	\$EPS	35	45	52
TEV	1,149,439	P/E	33.2	26.0	22.5

Our view is if LLY has at least modest upside to current consensus, we'll be at over \$75 earnings and a \$1,500 stock within a reasonable timeframe. Should some aspects of the bull case thesis play out, LLY should continue to have above-market growth for many years thereafter and as holders we will be well rewarded and will have played our small part in financing a revolution in beneficial healthcare outcomes.

I do not hold a position with the issuer such as employment, directorship, or consultancy.

I and/or others I advise hold a material investment in the issuer's securities.

Catalyst

Continued leadership in high-grwth incretin category.

Flywheel of portfolio improvements through acquisitions and partnerships

Upside to conservative forecasts. Proxy for AI drug development.

Life-enhancing, life-saving mission.

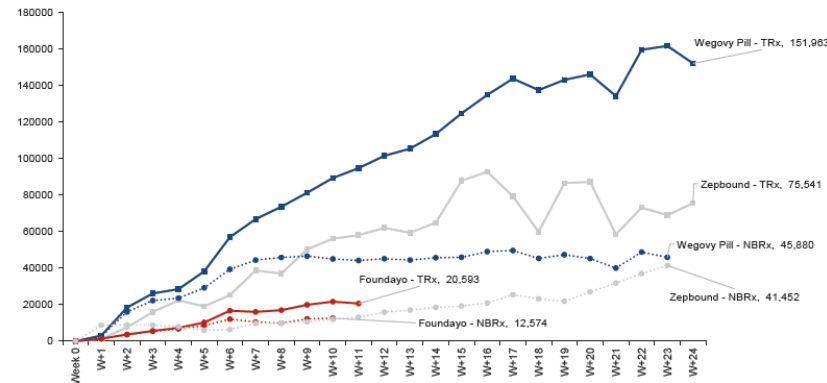
Messages

Subject Novo offers a better R/R
Entry 07/20/2026 03:46 PM
Member seb1991
 Imo, Novo offers a better R/R. Also:

"Again, I think it is pure noise that analysts are saying LLY's pill Foundayo had an "underwhelming launch" compared to Wegovy's pill ".

>> Calling it "underwhelming" does not reflect the reality. Novo's Wegovy TRx is ~8x larger than \$LLY. The below graph illustrates the development. To note, the wegovy pill numbers are also understated as IQVIA only captures ~60% of the total TRx volume, while ~90% for Foundayo.

EXHIBIT 3: TRx and NBRx in the initial weeks of launch for Zepbound, Wegovy Pill, Foundayo



Source: IQVIA, Bernstein

"LLY's higher efficacy products are set to dominate the market"

>> Are you not contracting yourself? The Wegovy pill has a higher % efficacy vs Foundayo, so your claim is invalid for the pill.

I would suggest to talk to physicians regarding the key purchasing criteria. Unlike what investor think, it is not efficacy (%) what matters the most but pricing/cost > insurance coverage > comorbidities benefits > efficacy. In that sense, Novo is on par with LLY.

Subject Re: Novo offers a better R/R
Entry 07/21/2026 11:47 PM
Member Shoe

Foundayo has fewer side effects, better tolerability, and easier administration. Wegovy is just a huge dose of semaglutide packed into a pill - which isn't a smart use of a somewhat scarce active ingredient. I think it does make sense that the script outperformance comes from Wegovy's had a head start and brand name. Lilly is turning on the marketing engine now too.

LLY mounjaro and zepbound are better than ozempic and wegovy. Just look at LLY's much larger share

And LLY's next gen drugs are also better. While Novo's pipeline has completely bombed. As you know, biopharma without a pipeline can stay very cheap

So LLY is better on current gen, next gen, and maybe orals. I think you stick with the winner

Subject Re: Re: Novo offers a better R/R
Entry 07/22/2026 05:37 AM
Member seb1991

Disagree with a lot of those comments.

1/ Foundayo has significant restrictions when taken together with statins. Yet ~40–45% of US adults with obesity are taking a statin, so Foundayo has a structural disadvantage. Your API scarcity claim is flawed because sema is not a scarce ingredient (it does not grow on trees) rather Novo is producing it at scale.

2/ LLY's Mounjaro and Zepbound are better than Ozempic and Wegovy - on what metric? In fact, the FDA and EMA have granted Novo broader comorbidity labelling benefits than LLY.

3/ The sell-side forecasted Foundayo to ramp up from Day 1 yet it is performing far below expectations. Novo also has stronger brand awareness as people are more familiar with the Wegovy franchise, while the majority of people cannot associate Foundayo with Lilly. Your claim that efficacy is key but then you dont apply it to the pill format.

I am aware that investors view NVO Cagrisema as inferior to LLY because the WL efficacy delta is like 2pp (23% vs 25%). However, the average consumer doesn't care about WL, in fact, most people don't want to lose 23-25% WL and are happy with ~15%. Again, price > comorbidity > efficacy.

Noting that today's market share is like 45/55%; not 10/90%.

Subject	Good Barron's article
Entry	07/31/2026 07:54 AM
Member	gary9
https://www.barrons.com/articles/glp-1-obesity-drugs-eli-lilly-novo-nordisk-stocks-19200dcc	

should have hired this guy to do my write up 💎